



Valuing the Hub Function

Crediting support for other institutions' research

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Where we stand

Not one hospital's site visit. We speak for a national system and ask how it should measure a research hub.

01 **UTokyo**

Director of the ARO at UTokyo Hospital, itself a Core Clinical Research Hospital.

02 **NUH-CRPI**

Representative Secretary of the council of all national university hospitals: 42 universities, 44 hospitals.

03 **J3C, AMED**

Vice Chair, Japan CDISC Coordinating Committee. AMED PI on data standards.

A year on from our last visit

Last October you asked whether our EDC would connect to the EHR. Our answer starts one step earlier.

- 01 At entry** Each EHR item gets its CDISC variable and terms (incl. MedDRA, WHODrug) at entry, via FHIR.
- 02 Then** Any EDC can use the data, REDCap included. The link to an EDC comes after.
- 03 So far** About 1,180 CDISC code lists as FHIR code systems; a terminology server in pilot.

Japan's Core Clinical Research Hospitals

A legal status, not a grant. The Minister approves it; 16 hospitals hold it today.

- 01 **Since 2015** Created in 2015. Approval and renewal follow published criteria.
- 02 **A status** Unlike a CTSA award, a status the hospital keeps until it fails the criteria.
- 03 **No office** No national program office sets goals. The criteria are the whole instrument.

What the criteria count

Three-year counts, in principle in-house, decide approval. Support to others is a floor, not a score.

01 **8 trials**

Drug-approval trials, or 4 plus 40 others. In principle led by the hospital's own PI.

02 **45 papers**

Peer-reviewed English papers. In principle the first author is from the hospital.

03 **15 supports**

A fixed floor. Beyond it, only full support counts: protocol, DM and monitoring.

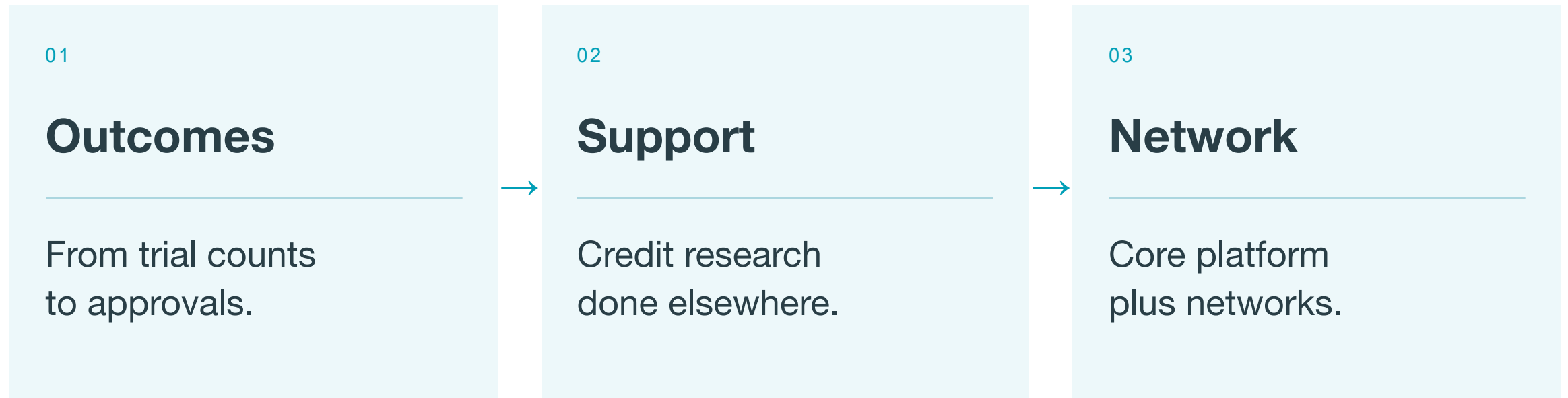
What happens as a result

Counting in-house output creates an incentive that works against the hub function.

- 01 **Small trials** Hospitals launch small, easy-to-start trials and multiply papers to meet the thresholds.
- 02 **Penalty** Every person or dollar moved to supporting others lowers the hospital's own metrics.
- 03 **Floor stays** The 2026 review keeps support as a floor; its effort was judged too hard to score.

Three shifts we propose

Re-weight, do not abolish. Each shift has a working US counterpart in how NIH funds hubs and functions.



Measure what a hub enables, not what it keeps in-house.

Measured by our own yardstick

Apply the proposed axis to ourselves. None of this counts today; all of it enabled others' research.

- 01 **Ptosh** EDC platform for academic trials; CDISC encoded at data entry.
- 02 **NHO** CDISC adopted across the National Hospital Organization's multicenter trials.
- 03 **aCRF.jp** Open annotated CRF library, and the JASPEHR group on EHR-to-research data.

What we came to ask

Eight written questions; your agenda gives each a session today.

01 This morning Community engagement (Q8); the Coordinating Center, TIN (Q3).

02 With Paul NCATS metrics, unfunded functions, post-cut mix, open infrastructure (Q1, 2, 4, 6).

03 Then Workforce with Terri (Q5); EHR and BioVU research support (Q7).

What we most want to learn

One question is about the review; two are specific to you.

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|----|-------------------|--|
| 01 | Review | What decided your last renewal; which report numbers has NCATS acted on? |
| 02 | If counted | If in-house counts decided status, could REDCap and TIN stay open? (Q6) |
| 03 | All of Us | Your largest award, a data center outside peer review: how is it judged? |



Thank you

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